

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 25

Age: 62 years old

Sex: Male

MRN: 000-TEST-25

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 3 days

****PRIMARY & SECONDARY DIAGNOSES****

- Type 2 Diabetes Mellitus with hyperglycemic episode (admission glucose 487 mg/dL)
- Chronic Obstructive Pulmonary Disease (COPD), moderate, on home oxygen
- Peripheral Arterial Disease (PAD), left lower extremity with claudication
- Hypertension
- Dyslipidemia

****HOSPITAL COURSE****

Patient 25 presented via ED with polydipsia, polyuria, and fatigue × 5 days. Labs on admission revealed glucose 487 mg/dL, HbA1c 9.2%, arterial pH 7.32 (mild metabolic acidosis). UA positive for ketones. No DKA; diagnosis: hyperglycemic crisis secondary to medication non-adherence and acute URI triggering increased glucose demand.

Treated with IV fluids (normal saline 1L bolus, then maintenance), insulin drip (initial 0.1 u/kg/hr, titrated to target 150–200 mg/dL). Glucose normalized by hospital day 2 (158 mg/dL). CXR showed mild hyperinflation, no acute process. EKG normal. Patient counseled on DM management, carbohydrate counting, sick day rules. Endocrinology consulted on day 2; recommended insulin-based regimen. COPD stable on current inhalers; no acute exacerbation. PAD asymptomatic during admission; ABI measurements pending outpatient vascular lab.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Insulin Glargine	20 units	q.d. at bedtime	ongoing
Insulin Lispro	8 units	before meals (TID)	ongoing
Metformin	1000 mg	BID with meals	ongoing
Lisinopril	20 mg	q.d.	ongoing
Atorvastatin	40 mg	q.d. at bedtime	ongoing
Albuterol MDI	2 puffs	q.4–6h PRN SOB	ongoing
Tiotropium DPI	1 inhalation	q.d.	ongoing
Aspirin	81 mg	q.d.	ongoing
Omeprazole	20 mg	q.d.	ongoing

****GLUCOSE MONITORING & MANAGEMENT****

- Home fingerstick glucose monitoring: fasting q.d. and before dinner daily; additional checks pre/post-exercise
- Patient instructed on insulin injection technique, pen storage, hypoglycemia recognition (tremor, sweating, confusion)

- Glucagon emergency kit prescribed; family member trained on administration
- Dietary consult completed: 2000 kcal ADA diet, carb counting education, alcohol restriction

****ACTIVITY & MOBILITY****

- Weight-bearing as tolerated, both lower extremities
- Avoid prolonged standing due to PAD; elevate legs when resting
- No strenuous exertion; walking 15–20 min q.d. as tolerated
- Continue home oxygen at 2L/min continuously during day; titrate SpO2 goal $\geq 88\%$

****FOOT CARE****

- Daily visual inspection of feet (dorsal/plantar surfaces, between toes) for blisters, erythema, cracks